

Department of Neuro Surgery
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 51.9 Yr/M
Admission No.: anonymous	Department Unit: Neuro Surgery (Ns Unit 1)	Consultant/Faculty: anonymous
Ward: 3 B 2 Neuro Surgery Ward	Room/Bed: 3 B 2 Neuro Surgery/ 3 B 2 N S 3	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 13-Mar-2023 10:52	D.O.D. : 05-Apr-2023	D.O. Print Report: 08-Feb-2026 15:44

FINAL DIAGNOSIS:

Cervical myelopathy;

Remarks: C3-C5 MIXED OSSIFIED POSTERIOR LONGITUDINAL LIGAMENT AND HYPERTROPHIED LIGAMENTUM FLAVUM WITH CANAL STENOSIS

OPERATION PERFORMED:

C3-C5 DECOMPRESSION LAMINECTOMY On 23-Mar-2023

SURGEONS: Dr Binoy Kumar Singh

OPERATIVE FINDINGS: INCISION GIVEN C1 TO C6 SPINOUS PROCESS FOLLOWED BY C3 TO C5 SUBPERIOSTEAL MUSCLE

DISSECTION GUTTER WAS MADE BILATERALLY FROM C3 TO C5, THEN LAMINA WAS REMOVED. HYPERTROPHIED LIGAMENTUM FLAVUM FROM C3-C5 EXCISED. CORD BECAME LAX AND PULSTILE. DURAL TEAR OF 0.5X0.5 CM OCCURRED AT LEVEL OF C4 VERTEBRA WHICH MANAGED BY APPLYING TISSUE GLUE. HEMOSTASIS ACHIEVED. INCISION CLOSED IN LAYERS.

HISTORY OF PRESENT ILLNESS: C/O STIFFNESS IN LOWER LIMB> UPPER LIMB SINCE 7-8 MONTHS

C/O CLUMSINESS OF BILATER HANDS SINCE 4-5 MONTHS

C/O PARESTHESIA IN B/L HANDS> FEET SINCE 4-5 MONTHS

C/O DIFFICULTY IN WALKING

C/O WEAKNESS IN RIGHT LOWER LIMB

PAST HISTORY: NOT SIGNIFICANT

CNS EXAMINATION: CONSCIOUS, ORIENTED TO TIME, PLACE AND PERSON. GCS- E4V5M6.

PUPIL-B/L SYMMETRICAL AND

EQUALLY REACTIVE TO LIGHT.

MOTOR- BULK- NO WASTING OR ANY FASCICULATION, TONE- NORMAL IN UPPER LIMB AND INCREASED IN BOTH LOWER LIMBS.

POWER- B/L UPPER LIMB-4 /5, GRIP: RT-60-70%, LT-80-90% LOWER LIMB- HIP- FLEXION/ EXTENSION- RT- 4/5, LT- 4 /5, KNEE- FLEXION/ EXTENSION- RT-4/5, LT- 4 /5, ANKLE- DF- RT- 4/5, LT- 4 /5, ANKLE- PF- RT- 4/5, LT- 4 /5, EHL- RT- 4/5, LT- 4 /5.

DTR - 3 IN LIMBS, PLANTAR- BILATERAL EXTENSOR.

NO SENSORY DEFICIT PRESENT.

BOWEL AND BLADDER: NOT INVOLVED.

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
22-Mar-2023 18:30	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.01	0.02 - 0.10 x10 ³ /μL

	Absolute Eosinophil Count	0.01	0.02 - 0.5 x10 ³ /μL
	Absolute Lymphocyte Count	1.34	1 - 3 x10 ³ /μL
	Absolute Monocyte Count	0.04	0.2 - 1.0 x10 ³ /μL
	Absolute Neutrophil Count	10.05	2 - 7 x10 ³ /μL
	Basophil	0.1	00 - 01 %
	Eosinophils	0.1	01 - 06 %
	HCT	44.5	40 - 50 %
	Hemoglobin	14.6	13 - 17 g/dl
	Lymphocytes	11.7	20 - 40 %
	MCH	28.4	27 - 32 pg
	MCHC	32.8	31.5 - 34.5 g/dL
	MCV	86.6	83 - 101 fL
	Monocytes	0.3	2 - 10 %
	Neutrophils	87.8	40 - 80 %
	Platelet Count	203	150 - 410 x10 ³ /μL
	RBC	5.14	4.5 - 5.5 x10 ⁶ /μL
	Red Cell Distribution Width (RDW CV)	11.8	11.6 - 14.0 %
	Total Leukocyte Count(TLC)	11.45	4 - 10 x10 ³ /μL
Liver Fuction Test (Lft)	Albumin	3.9	4.0 - 5.0 gm/dL
	ALP	66	50 - 116 U/L
	ALT	41	7 - 41 U/L
	AST	31	12 - 38 U/L
	Direct Bilirubin	0.22	0.1 - 0.4 mg/dl
	globulin	3.6	2.0 - 3.5 gm/dL
	Indirect Bilirubin	0.77	0.20 - 0.90 mg/dl
	Total Bilirubin	0.99	0.3 - 1.3 mg/dl
	Total Protein	7.5	6.7 - 8.6 gm/dL
	Chloride	100	102 - 109 mmol/L
Rft	Creatinine	0.96	0.9 - 1.3 mg/dl
	Potassium	3.82	3.5 - 5 mmol/L
	Sodium	136	135 - 145 mmol/L
	Urea	17	15.0 - 39.0 mg/dl

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: MRI DORSO LUMBAR SPINE (15/2/2023): PLL HYPERTROPHY AT C2-C3 AND C4-5 LEVEL, CORD SHOWS T2 HYPERINTENSITY AT C3-4, C5-C6 LEVEL S/O MYELOMALACIA.

CT DL SPINE S/O OSIFIED POSTERIOR LONGITUDINAL LIGAMENT AT C2-3, C4-5 LEVEL

HOSPITAL COURSE: PATIENT CAME WITH ABOVE MENTIONED CHIEF COMPLAINTS AND WAS DIAGNOSED AS A CASE OF C3-C5 MIXED OSSIFIED POSTERIOR LONGITUDINAL LIGAMENT AND HYPERTROPHIED LIGAMENTUM FLAVUM WITH CANAL STENOSIS, FOR WHICH PATIENT UNDERWENT C3-C5 DECOMPRESSION LAMINECTOMY UNDER GA ON 23/3/23. POST OPERATIVELY PATIENT HAS WOUND DISCHARGE CSF LEAK FOR WHICH ANTIBIOTICS WERE CONTINUED AND WAS MANAGED CONSERVATIVELY. REST PATIENT



RECOVERED WELL. HENCE, PATIENT IS NOW BEING DISCHARGED WITH STABLE VITALS.

CONDITION AT DISCHARGE: PATIENT CONSCIOUS, ORIENTED WITH ALL STABLE VITALS, GCS- 15/15. TONE- NORMAL IN UPPER LIMB AND INCREASED IN BOTH LOWER LIMBS.

POWER- B/L UPPER LIMB-4 /5, GRIP: RT-60-70%, LT-80-90% LOWER LIMB- HIP- FLEXION/ EXTENSION- RT- 4/5, LT- 4 /5, KNEE- FLEXION/ EXTENSION- RT-4/5, LT- 4 /5, ANKLE- DF- RT- 4/5, LT- 4 /5, ANKLE- PF- RT- 4/5, LT- 4 /5, EHL- RT- 4/5, LT- 4 /5.

L/E: SUTURES IN SITU, WOUND HEALTHY

ADVICE ON DISCHARGE:

- DAILY BATH, MAINTAIN HYGIENE.
- HARD CERVICAL COLLAR APPLICATION
- AVOID LIFTING HEAVY WEIGHTS
- TAB AUGMENTIN 625 MG TDS FOR 5 DAYS
- TAB PANTOPRAZOLE 40MG OD EMPTY STOMACH FOR 5 DAYS.
- TAB GABAPENTIN NT (400/10) MG HS FOR 30 DAYS
- TAB ETORICOXIB THIOCOLCHICOSIDE (60 4MG) BD FOR 7 DAYS
- TAB LIOFEN XL 30MG BD FOR 7 DAYS
- SYP LACTULOSE 30ML HS FOR CONSTIPATION

DISCHARGE BY : anonymous

DISCHARGE REMARKS : IN CASE OF ANY EMERGENCY LIKE WOUND INFECTION/DISCHARGE, WEAKNESS REVIEW TO EMERGENCY TRAUMA OR IN NEUROSURGERY OPD.

FOLLOW UP ON : 11-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : REVIEW AFTER 1 WEEK TO NEUROSURGERY OPD.

Summary Prepared By

Summary Approved By

anonymous

anonymous